# 6 Wound Dressing (Aseptic Technique)

HME922 Procedural Station · 10 minutes · standard ANTT (check against wound guideline)

guideline wording — say it aloud  DOPS On the examiner's DOPS marking form  ☐ tick when done

**KEY MESSAGE**

Protect **key parts and key sites** with aseptic non-touch technique. Assess the wound systematically, clean **clean → dirty**, match dressing to wound, document.

## UNIVERSAL OPENING

- ☐ Hand hygiene — Moment 1
- ☐ Introduce yourself: name + "medical student"
- ☐ Confirm ID — **3 identifiers**
- ☐ Explain why the dressing is being changed
- ☐ Ask about **allergies** (tape, iodine, chlorhexidine, latex) + pain/analgesia (give 30 min before)
- ☐ Allow questions
- ☐ Gain **informed verbal consent**
- ☐ Privacy, lighting, expose only the wound, bed height up

**MEMORY AID — 5 MOMENTS**

**5 Moments of Hand Hygiene:** 1 before touching patient · 2 before procedure · 3 after body-fluid risk · 4 after touching patient · 5 after touching surroundings

## CLINICAL KNOWLEDGE — SAY IT

- ☐ Purpose: protect from infection/trauma, absorb exudate, moist healing environment, comfort
- ☐ Change when: care plan due, soiled/lifting/strike-through, signs of infection
- ☐ Infection signs: spreading redness, heat, swelling, pain, pus, odour, breakdown, fever

## PREPARE

- ☐ Clean trolley (detergent/alcohol wipe, dry)
- ☐ Dressing pack, sterile 0.9% saline, sterile + non-sterile gloves, dressing, tape, waste bag, measuring guide, bluey
- ☐ Check **expiry** and packaging intact/dry

## PROCEDURE

1. Hand hygiene → non-sterile gloves → remove old dressing; inspect it → discard with gloves
2. Hand hygiene → open pack by outer corners; drop saline + dressing onto field
3. Sterile gloves / forceps — **key parts touch nothing non-sterile**
4. **Assess:** site · size (L × W × D) · bed (granulation, slough, necrosis) · exudate · edges · surrounding skin · odour · pain
5. Clean **clean → dirty**; one swab, one stroke, discard
6. Pat surrounding skin dry
7. Apply dressing with 2–3 cm margin; secure without tension

**MEMORY AID — WOUND ASSESSMENT — "TIME"**

**T**issue (bed) · **I**nfection / inflammation · **M**oisture (exudate) · **E**dges



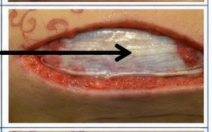
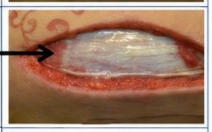

## UNIVERSAL CLOSING

- ☐ Sharps to sharps bin — never recap
- ☐ Waste sorted; trolley cleaned
- ☐ Area clean; patient comfortable; bed lowest; call bell
- ☐ Report pain, bleeding, fever, redness, soaked dressing
- ☐ Hand hygiene — Moments 4/5
- ☐ Document assessment, dressing, next review; date on dressing
- ☐ Escalate infection/deterioration
- ☐ Can't answer? "I'll pass that on to someone senior"
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

### Wound Bed Structures

SKIN & WOUND – QUICK REFERENCE GUIDE

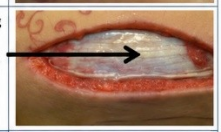
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| Type of Tissue | Appearance | Example Photo |
|---|---|---|
| Fibrin | • Yellow tissue, firmly attached to wound bed, cannot be picked up with forceps.<br>• Indicates normal healing.<br>• Precursor to granular tissue and should not be debrided. |  |
| Adipose | • Creamy, yellow.<br>• Wet and soft when healthy.<br>• Black/brown and can dry out when unhealthy. |  |
| Fascia | • Connective tissue, shiny white, thin to thick depending on location.<br>• Encapsulates structures such as nerves, bone, vessels, muscles and organs.<br>• Should be shiny and wider than a tendon, as it wraps around the structure. Not as firm as tendon or bone. |  |
| Muscle | • Striated, pink to dark red.<br>• Firm and highly vascular. |  |
| Tendon | • When healthy, appears as strong fibrous tissue, shiny white or yellow, attaches muscle to bone.<br>• Unhealthy tendon may yellow and can appear like slough.<br>• Should have attachment points on bone and muscle. If the client moves the nearest joint, the tendon should shift or move. |  |
| Bone | • Hard, shiny, smooth, white appearance when healthy.<br>• When unhealthy, can be brittle, rough and discoloured.<br>• Often found in DFU and pressure injuries.<br>• Will make a scraping sound when an instrument comes in contact with it.<br>• If bone is palpable or visible, there is an increased risk of osteomyelitis, which can be very difficult to treat. |  |

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